



ROOT CAUSE UNLOCKED · CONDITION KIT

The Allergy Root Cause *Kit*

Which tests actually diagnose an allergy, which ones do not, and exactly what to ask your doctor for

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The Allergy Root Cause Kit

Which tests actually diagnose an allergy, which ones do not, and exactly what to ask your doctor for

BEFORE YOU START

Contents

Read this part first

The one rule that explains most confusing results

How to read this guide

Tier 1: the tests that actually diagnose allergy

Tier 2: the terrain around the allergy

Tier 3: when the picture does not fit

Food-specific IgG panels: what they are for, and what they are not for

Tests sold to allergy patients that do not do what the marketing says

Reading your results as a pattern

The conversation with your doctor

What these labs cannot tell you

When to seek clinical review

Your next step

One thing no panel on this page measures

References

Read this part first

Diet does three different jobs here, and mixing them up is the whole problem

The other half of avoidance nobody mentions

What is claimed

What happened when it was tested directly

Where dietary histamine did show something

What I do with all of this

Running the four-week trial properly

Two popular ideas that did not survive testing

A week that is actually cookable

What I deliberately left out, and why

When to seek professional care

Working with Root Health

References

Read this part first

Why nutrient evidence looks thinner than it is

Why the bar is higher in this guide than in most of mine

How to read this guide

Quercetin and related polyphenols

Butterbur extract (*Petasites hybridus*)

Spirulina

Probiotics

Vitamin D, if yours is actually low

DAO enzyme, and the cofactors it runs on

Stinging nettle (*Urtica dioica*)

Vitamin C

Bromelain

What I deliberately left out, and why

The non-supplement purchases people make, judged the same way

Running a fair trial when the pollen count is moving

Realistic expectations

When to seek professional care

Where to find quality versions of these

Working with Root Health

The nervous system, which sets how loudly you feel all of this

References

Read this part first

This guide is educational. It is not medical advice, it does not diagnose anything, and reading it does not make you my patient. Bring any result that concerns you to a clinician who can see the whole picture.

Call 911 now, do not read further, if you are having a reaction with any of these: trouble breathing, throat tightness or a hoarse voice, swelling of the lips or tongue, vomiting with hives, dizziness or fainting. That is anaphylaxis. If you have been prescribed epinephrine, use it first and then call. Anaphylaxis is not something to test your way through.

Allergy is unusual in the wellness world, and it works in your favor. Most conditions I write about have no validated diagnostic test, which is why so much interpretation is judgment. Allergy is not one of them. Real allergy has real tests, they have been validated against controlled food and allergen challenges, and they are widely available and usually covered by insurance. That means two things. You can get a genuine answer. And the tests marketed hardest to allergy sufferers online are, with few exceptions, not the ones that give it.

The one rule that explains most confusing results

Sensitization is not allergy.

A positive skin prick test or a positive blood IgE result tells you that your immune system has made IgE antibodies to something. It does not tell you that you react to it. Plenty of people carry IgE to foods and pollens they eat and breathe with no symptoms at all.

Allergy is the combination of two things: a positive test **and** consistent symptoms on real-world exposure. Neither half is a diagnosis on its own.

This rule is the reason a broad panel ordered without a history is a trap. Test forty allergens in someone with no story, and you will find positives. Those positives will then get treated as instructions, and the person will remove foods they were never reacting to. The test did not lie. It was asked the wrong question.

So the order matters, always:

1. **The history comes first.** What happens, how fast, after what exposure, how reproducible.
2. **The test is chosen to confirm or refute that specific story.**
3. **The result is interpreted against how likely the story was in the first place.**

A test result changes the odds of something. It does not replace the thing it is changing the odds of.

How to read this guide

Tests are grouped in three tiers plus a fourth list.

Tier 1 diagnoses allergy. If your question is “am I allergic to this,” the answer is here.

Tier 2 is the terrain around the allergy: nutrient status, inflammation, and blood counts. These do not diagnose anything allergic. They tell you whether the ground the immune system is standing on is solid, and they occasionally uncover something that has nothing to do with allergy and matters more.

Tier 3 is for when the picture does not fit: reactions without a positive allergen test, reactions to everything, symptoms that are systemic rather than nasal.

The fourth list is tests sold to allergy patients that do not do what the marketing says. Knowing what not to buy is worth as much as knowing what to buy, and it is cheaper.

Where I give a functional range, it is a clinical target drawn from the Root Health blood chemistry interpreter, not a validated diagnostic threshold, and it sits inside the standard laboratory range rather than replacing it. Your lab’s reference range is what your clinician will use, and it is printed on your report for a reason.

Tier 1: the tests that actually diagnose allergy

Skin prick testing

What it is. A drop of allergen extract is placed on the forearm or back and the skin is pricked through it. A raised wheal after fifteen to twenty minutes means IgE bound to mast cells in your skin recognized that allergen.

What it is good for. Fast, inexpensive, done in one visit, and it lets you and the clinician look at the result together. For airborne allergens in particular, it is a reasonable first-line test.

What can distort it. Antihistamines suppress the wheal, so they generally have to be stopped for several days beforehand. Ask exactly how long, because it differs by drug, and do not stop a prescription without saying so. Extensive eczema on the test area, some other medications, and very young or very old skin all change the reading. Dermatographism, where the skin welts from scratching alone, produces false positives and the controls on the test are there to catch it.

How to read it. Bigger wheal means more IgE bound. It does not mean a more severe reaction is coming. Wheal size tracks the probability that a reaction exists, not the severity of the reaction you will have. People are told the opposite constantly.

Serum specific IgE

What it is. A blood test measuring IgE antibodies against individual allergens. You may see it called by the assay's brand name on your report.

When it is the better choice. When you cannot stop antihistamines, when your skin is not testable, when there is a history of a severe reaction, or when you need a number to follow over time.

How to read it. The result comes in kU/L, and higher values raise the probability that a real clinical allergy is present. Formal guidance on food-specific IgE is explicit that a detectable level establishes sensitization but is not always accompanied by clinical reactivity, and that age, the pattern of symptoms, coexisting conditions such as eczema or pollinosis, and how the person was selected for testing all change what a given number means (PMID 20128415). The same guidance covers the harder questions this test is actually used for: whether a food can be reintroduced, and what the number predicts over time.

Translation: the number is only interpretable next to your history. A level of 2 kU/L in someone who swells reliably after eating that food means something entirely different from the same number in someone who eats it weekly without a symptom.

Component testing. For some foods, the lab can measure IgE against individual proteins within the food rather than the whole extract. This can separate the person who reacts to a heat-stable protein, and is at

genuine risk, from the person reacting to a heat-labile pollen-cross-reactive protein, who typically gets an itchy mouth from raw fruit and nothing worse. If your reactions are limited to raw produce, this is worth asking about by name.

Total IgE

What it is. All the IgE in your blood added together, not directed at anything in particular.

What it is good for. Very little on its own. It can be elevated in allergy, and also in parasitic infection, some immune conditions, and in smokers. It can be normal in someone with a clear, dangerous, single-allergen allergy. It is sometimes used to help interpret specific IgE results or to qualify for certain biologic medications.

Do not use it as an allergy screen. A normal total IgE does not rule allergy out, and a high one does not tell you what you are allergic to. It gets ordered as a screen constantly, and it answers almost nothing by itself.

The test that beats all of them, which is not a lab test

For food, the diagnostic reference standard is a supervised oral food challenge: you eat the food, in graded amounts, in a setting equipped to treat a reaction. Blood and skin testing exist largely to decide who needs a challenge and who does not (PMID 20128415). If you have been avoiding a food for years on the strength of a panel result and no clear reaction history, the question worth asking an allergist is whether a supervised challenge is appropriate. People get foods back that way.

Never do this at home with a food that has ever caused a serious reaction.

Tier 2: the terrain around the allergy

None of these diagnose an allergy. They are worth running because they occasionally explain fatigue or symptoms that were being blamed on allergy, and because two of them, vitamin D and iron, are common, cheap to correct, and have downstream effects on how you feel.

Complete blood count with differential

Eosinophils are the white cell most associated with allergic inflammation. They rise in allergy, and also in parasitic infection, some drug reactions, and a set of uncommon conditions that need investigating rather than supplementing.

Read the **absolute** eosinophil count, not only the percentage. A percentage moves when the other cell lines move, so a “high” percentage on a low total white count can be unremarkable, and a normal-looking percentage on a high white count can hide a real elevation.

A mildly raised eosinophil count fits an allergic picture and does not need chasing on its own. A markedly or persistently raised count is a reason to see a physician, not a reason to buy anything. Travel history and parasite exposure matter here.

Basophils are the other cell in the histamine story, but the blood count is a poor window on them and it is frequently reported as zero in perfectly healthy people. Do not read much into it.

The rest of the differential is worth a look for a different reason: it can quietly show an infection, an anemia, or something that needs a physician, in a person who booked the visit about their nose.

Vitamin D, 25-hydroxy

Worth testing, worth correcting if low, and worth being honest about what correcting it does.

The observational association is real: lower 25-hydroxyvitamin D levels track with allergic rhinitis prevalence and severity in a number of studies. The treatment evidence is weaker and mixed, with benefit appearing to depend on baseline level, age, sex, and what else the person is taking (PMID 41809154).

The two supplementation trials worth knowing are both in people who were **already deficient**. In a randomized double-blind placebo-controlled trial, 80 patients with allergic rhinitis and vitamin D deficiency received cetirizine plus either vitamin D or placebo. Mean baseline level was about 14 ng/mL, the supplemented group reached about 24 ng/mL, and symptom severity showed no significant difference at four weeks but a significant difference by eight weeks (PMID 31332549). A separate randomized controlled trial adding vitamin D3 to an antihistamine for four weeks reported lower symptom scores, lower serum IL-4, and lower peripheral blood eosinophils in the combination group compared with the antihistamine alone (PMID 33382385).

Read that as: correcting a real deficiency is worth doing, the effect is modest and takes weeks, and none of it argues for high-dose vitamin D in someone whose level is already fine.

Iron studies and ferritin

Iron deficiency causes fatigue that gets misattributed constantly, including to allergies and to antihistamines. Ferritin alone is not enough, because it rises with inflammation and can look reassuring in someone who is genuinely low. Ask for ferritin with serum iron, TIBC, and transferrin saturation, alongside the CBC.

Inflammation

hs-CRP is a general marker. It is useful as a background reading and it is not an allergy test. Allergic inflammation is local and IgE-driven and often runs with a perfectly normal CRP. A high CRP in someone with nasal symptoms points toward infection or something systemic, not toward a worse allergy.

The nutrients behind histamine metabolism

Two enzymes clear histamine. DAO works on histamine in the gut, and its activity depends on vitamin B6, copper, and vitamin C as cofactors. HNMT works inside cells and uses a methyl donor, which is why B12, folate, and homocysteine get attention in this conversation.

Be clear about what this reasoning is and is not. It is cofactor biochemistry, and it is a legitimate reason to check whether you are depleted in something. It is not evidence that supplementing these nutrients improves allergy symptoms in people who are not deficient. I have not found trials showing that, and I looked. Test them because a real deficiency is worth correcting on its own terms.

Functional ranges for the terrain markers

These are clinical targets used at Root Health, not validated diagnostic thresholds, and they are narrower than the standard laboratory range on purpose. Being outside a functional range is a prompt to look closer. It is not a diagnosis.

Marker	Units	Functional target	Approximate standard range
White blood cells	K/uL	5.0 to 7.5	3.5 to 10.5
Neutrophils	%	50 to 70	40 to 75
Lymphocytes	%	25 to 40	15 to 45
Monocytes	%	4 to 9	up to 12
Eosinophils	%	1 to 4	up to 7
Basophils	%	0 to 1.5	up to 2.5
Vitamin D (25-OH)	ng/mL	40 to 80	20 to 100
Ferritin	ng/mL	50 to 150	25 to 300
Serum iron	ug/dL	65 to 110	40 to 150
TIBC	ug/dL	290 to 370	250 to 400
Transferrin saturation	%	20 to 40	15 to 45
hs-CRP	mg/L	under 0.5	under 1.0
Vitamin B12	pg/mL	500 to 900	200 to 1100
Folate (RBC)	ng/mL	600 to 1500	400 to 2000
Homocysteine	umol/L	under 7.0	under 10.0
Vitamin B6 (P5P)	nmol/L	30 to 100	20 to 120
Zinc (plasma)	ug/dL	70 to 110	60 to 120
Copper (serum)	ug/dL	80 to 130	70 to 160
Magnesium (RBC)	mg/dL	5.2 to 6.8	4.0 to 7.2
Magnesium (serum)	mg/dL	2.0 to 2.5	1.5 to 2.7
Albumin	g/dL	4.2 to 4.8	3.5 to 5.0

A note on the last one. Albumin is on this list because it is the cheapest sanity check on the rest of the panel. Several markers ride on it, and a low albumin quietly changes how the others read.

Tier 3: when the picture does not fit

Some people have real, reproducible, sometimes frightening reactions and every allergy test comes back negative. That is a recognizable situation and it has its own workup. It is also the situation with the most confident misinformation attached to it, so this section is deliberately careful.

Baseline serum tryptase

What it is. Tryptase is a mast cell enzyme. A level drawn when you are well is your baseline. A level drawn within a few hours of an acute reaction, compared against that baseline, is a different and more informative test.

Why it is worth knowing. An elevated baseline tryptase, generally above 8 ng/mL, is most often explained by hereditary alpha-tryptasemia, a common genetic trait caused by extra copies of the TPSAB1 gene, diagnosed by tryptase copy number testing (PMID 41801525). That review is candid that the relationship between this trait and mast cell disease is still contested, with some studies suggesting an increased risk of severe anaphylaxis and others failing to reproduce consistent associations.

Why it matters practically. It reframes a person who has been told their symptoms are unexplained, and it changes the threshold for carrying epinephrine in some cases. That is a physician conversation.

About mast cell activation syndrome

This diagnosis is applied far more often than it is met. The consensus criteria require typical episodic symptoms in at least two organ systems, a substantial rise in serum tryptase above the person's own baseline during an event, and a response to mast-cell-directed treatment. A critical review in the allergy literature compares the consensus criteria against the alternative criteria circulating in practice and concludes that the alternatives often lack specificity and validation, and that an increasing number of patients are receiving a misdiagnosis on the strength of them (PMID 34166845).

I am not telling you your symptoms are not real. I am telling you that a diagnosis handed over without a tryptase drawn during an actual event is not a diagnosis, and that accepting it closes off the search for what is really happening. If this label has been applied to you, the useful next question is which specific criteria were met.

Serum DAO activity

Why this test is on my list. DAO is the enzyme that breaks down histamine you eat, working in the lining of the small intestine before that histamine is absorbed. It is the front line of histamine clearance from food, and its activity depends on vitamin B6, copper, and vitamin C as cofactors. When DAO capacity is low, dietary histamine gets through in amounts a person with normal DAO would have handled without noticing. That

mechanism is the core of the histamine picture and it is why I test the enzyme rather than guessing about the symptom.

Gut damage matters here as much as genetics. The cells that make DAO live in the intestinal lining, so inflammation, dysbiosis, and chronic gut injury reduce DAO output regardless of what your genes say. That is also why the histamine picture so often improves when the gut is addressed rather than only the diet.

How I read the number. A low DAO in someone whose symptoms fit is a meaningful finding and worth acting on. Treat it as a strong input rather than a verdict, for one specific reason: in a crossover study, 18 people diagnosed with histamine intolerance were randomized to a low-histamine diet or an ordinary mixed diet with serum DAO measured after each phase, and the changes did not differ meaningfully between the diets, with 10 of 18 higher after the low-histamine phase and 8 lower or unchanged (PMID 38769188). The practical lesson from that is narrow and useful: **DAO is a baseline measure, not a progress measure.** Do not re-draw it to decide whether your diet is working. Use symptoms for that.

A normal DAO does not clear you either, since intracellular clearance runs through a second enzyme, HNMT, which uses a methyl donor rather than the DAO cofactors. That is the reason B12, folate, and homocysteine are on the terrain panel above.

What to ask for. Serum DAO activity, drawn at baseline before you change your diet, alongside B6, copper, zinc, B12, folate, and homocysteine, so that if DAO is low you can see whether a correctable cofactor shortfall is part of the reason.

Celiac serology and thyroid

Two cheap tests that catch things allergy panels miss. Celiac disease produces gut and skin symptoms that get attributed to food sensitivity, and the serology must be drawn while you are still eating gluten or it is uninterpretable, which is the single most common way this test gets wasted. Thyroid autoimmunity travels with chronic hives often enough that thyroid antibodies are reasonable when hives are the main problem.

Food-specific IgG panels: what they are for, and what they are not for

This is the test people ask me about most, and it is the one most often described in absolute terms by both sides. Neither absolute is right, so here is how I actually use it.

What it measures. IgG antibodies against individual food proteins, usually across a large panel, reported as a graded list. IgG is a different antibody class from IgE and it maps a different thing: delayed, dose-related reactivity patterns rather than immediate allergic reactions.

Why I run it. For the patient with diffuse, delayed symptoms and no IgE allergy, the hardest practical problem is where to start. There are hundreds of foods, symptoms lag hours to days behind the meal, and a blind elimination diet is a long, miserable, low-yield project. A panel narrows a field of hundreds down to a handful worth testing properly. In my clinical experience it is a genuinely useful starting map, and the patients who get somewhere with food are usually the ones who had a place to begin.

The two rules that make it work.

1. **The panel generates hypotheses. The reintroduction generates the answer.** Remove a flagged food, watch, put it back, watch again. A food that reproduces the symptom on reintroduction is a real finding for you, and a food that does nothing goes back in your diet permanently. The Allergy Diet Guide walks through exactly how to run that.
2. **Do not remove twenty foods at once and stay there.** That is where this test goes wrong. Long unnecessary avoidance narrows nutrition, makes eating socially difficult, and leaves you no wiser about which food mattered.

What it is not, and this part is a safety line rather than an opinion. An IgG panel is **not** an IgE allergy test and it cannot rule an allergy in or out. If you have a diagnosed IgE food allergy, a low IgG result for that food changes nothing about your avoidance plan. Never use a low IgG value as a reason to eat something that has ever caused a serious reaction. Those are two different antibodies answering two different questions.

Also worth knowing before you spend the money. IgG to a food is in part a normal response to eating it, so a food you eat daily is more likely to appear on the list than one you rarely touch. Read the results with that in mind. And there is a real professional disagreement here: the allergy literature reviewing unproven diagnostics does not consider food-specific IgG and IgG4 testing to be a validated diagnostic method (PMID 39022151). I am telling you that plainly, because you should know what the state of the published evidence is when you decide. My position is that its value is as a starting map for a structured elimination and reintroduction, not as a diagnosis, and that used that way it earns its place.

Tests sold to allergy patients that do not do what the marketing says

Zonulin and the intestinal permeability panels

Serum zonulin measured by commercial kit failed to correlate with the physiologic lactulose-mannitol measure of gut permeability in a study of healthy first-degree relatives of Crohn's disease patients (PMID 33841181). That is one study with 39 participants and one kit, so it is a caution rather than a refutation, but it means an elevated zonulin is suggestive at best.

On the broader barrier panels, the markers are not interchangeable. IgA anti-actin is the best validated of them, at 80% sensitivity and 85% specificity, and that validation is specifically for celiac disease with villous atrophy rather than for permeability in general (PMID 17652043). Read those panels as a pattern, and be skeptical of any single marker presented as proof.

Large environmental panels ordered without a history

Not a scam, just misused. Covered above under the sensitization rule: a broad panel run without a story generates positives that get treated as instructions.

Reading your results as a pattern

Three patterns cover most of what walks in.

Pattern one: clean allergy. Symptoms are seasonal or exposure-linked, specific IgE or skin prick is positive to the matching allergen, eosinophils are mildly up, everything else is unremarkable. This is the good outcome, because the treatment path for it is well established and effective, and because allergen immunotherapy can change the underlying disease rather than only masking it.

Pattern two: allergy plus a depleted terrain. The allergy tests are positive and so is a low vitamin D, a low ferritin, or both. Both halves are worth addressing. Expect the allergy treatment to do the work on the nasal symptoms and the terrain correction to do the work on the fatigue. Attributing all of the fatigue to the allergy is how a treatable anemia gets missed for a year.

Pattern three: symptoms without allergy. Reactions are real, allergen testing is negative. This is where non-allergic rhinitis, reflux, medication side effects, chronic urticaria, and the histamine question live. It is also the pattern most likely to be handed an expensive unvalidated panel. The right next moves are the Tier 3 tests and a careful symptom diary, in that order, and the Allergy Diet Guide covers the dietary half.

The conversation with your doctor

How to open it

Lead with the story, not the test you want. “For the last three springs I have had six weeks of sneezing, itchy eyes, and congestion starting in April, and over-the-counter antihistamines are only partly helping” gets you further than “I would like an allergy panel.” The first sentence tells them what to test for. The second asks them to guess.

If you want testing, the sentence that works is: “I would like to know specifically what I am reacting to, because I am considering immunotherapy.” That is a concrete clinical reason, and it usually gets a concrete answer.

A request list you can bring

Reasonable to ask for, in most primary care settings:

- CBC with differential, including absolute eosinophil count
- 25-hydroxy vitamin D
- Ferritin, serum iron, TIBC, transferrin saturation
- hs-CRP
- Celiac serology, if there are gut symptoms, drawn while still eating gluten
- Thyroid function and thyroid antibodies, if chronic hives are the main problem

Reasonable to ask for, often via referral to an allergist:

- Skin prick testing or serum specific IgE, targeted to your actual exposure history
- Component-resolved testing, if reactions are limited to raw fruits and vegetables
- Baseline serum tryptase, if reactions are systemic, or involve more than one organ system, or have ever required epinephrine
- A discussion of supervised oral food challenge, if you are avoiding a food based on a panel rather than a reaction

Usually ordered through a functional medicine practice rather than primary care, and often out of pocket:

- Serum DAO activity, with B6, copper, zinc, B12, folate, and homocysteine alongside it, if the histamine picture fits
- A food-specific IgG panel, if your symptoms are delayed and diffuse and you want a starting map for a structured elimination and reintroduction rather than a blind one

If your request is declined

Ask what the reasoning is and write it down. Sometimes it is good reasoning you had not heard. If the answer is that the test will not change management, that is a fair answer, and the follow-up question is what would change management. If you still want a test that is not indicated, most of these are available to order directly and out of pocket, and you should know the cost before you order rather than after.

Practical notes on getting drawn

Ask whether to fast, since the iron studies prefer a morning draw. Ask how long to hold antihistamines if you are having skin testing, and confirm which of your medications count. Get the actual numbers, not a portal message saying “normal,” and keep them, because the second set is far more informative than the first.

What these labs cannot tell you

- **How severe a future reaction will be.** Wheal size and IgE level track probability, not severity.
- **Whether a positive result is causing your symptoms.** That is what history and challenge are for.
- **Whether a supplement is working.** Nothing on this panel is a good progress measure for a supplement trial. Symptom scores are.
- **Why you feel bad if everything is normal.** A clean panel is information, not dismissal. Sleep, reflux, non-allergic rhinitis, medication effects, and chronic stress all produce these symptoms with normal labs.

When to seek clinical review

Urgently, and this is the section that matters most:

- Any reaction with breathing difficulty, throat tightness, tongue or lip swelling, repeated vomiting, or fainting. Call 911. Use epinephrine first if you have it.
- Wheezing or shortness of breath that is new or worsening.
- Hives that keep coming back for more than six weeks, especially with joint pain, fever, or weight loss.

Soon:

- A markedly or persistently elevated eosinophil count.
- Symptoms that started after beginning a new medication.
- Nasal symptoms on one side only, or bloody discharge, or loss of smell that does not recover.
- Any reaction that has ever needed emergency care, whether or not it was called anaphylaxis.

Your next step

Testing tells you what you are dealing with. It does not tell you what to do on Monday. The two companion guides in this kit do that: the **Allergy Diet Guide** covers what food can and cannot change, including an honest read of the histamine question, and the **Allergy Supplement Guide** covers what has actually been studied, at what dose, and which popular products failed their trials.

If you want the interpretation done against your history rather than against a general guide, that is what a consultation is for:

Book a free Root Discovery Session: rootcauseunlocked.com/discovery?utm_source=guide&utm_medium=ebook&utm_campaign=allergies-labs

Consultations are available to people located in North Carolina, where Dr. Seay is licensed.

If you are outside North Carolina, start a symptom and exposure diary. For allergy specifically, a diary is close to diagnostic: dates, symptoms, pollen counts, what you ate, where you slept. Patterns that are invisible from inside a bad week are obvious on paper across a season:

Free symptom tracker: rootcauseunlocked.com/tracker?utm_source=guide&utm_medium=ebook&utm_campaign=allergies-labs

One thing no panel on this page measures

Symptom burden is not the same as allergen burden. Two people with identical skin prick results can have very different lives, and the difference lives partly in sleep, stress load, and how much attention the symptoms command.

Psychiatric comorbidity, especially anxiety and depression, is highly prevalent in chronic spontaneous urticaria, and the authors of that work argue for integrating routine psychiatric assessment and stress management into standard care (PMID 41842758). A 2026 review describes a skin-brain-exposome axis in stress-sensitive inflammatory skin conditions and makes the same argument (PMID 42074836). That evidence is strongest in urticaria and skin disease rather than in allergy generally, which is why I have named the conditions.

There is no test for this on your panel. The Allergy Supplement Guide covers what to do about it.

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Educational content only. Not medical advice. Not a diagnosis. Not a substitute for your clinician. Lab interpretation belongs with someone who can see your full history.

Read this part first

This guide is educational. It is not medical advice, it does not diagnose anything, and reading it does not make you my patient.

If you have a diagnosed IgE food allergy, nothing in this guide changes your avoidance plan. Do not reintroduce a food that has ever caused a serious reaction, and do not run any home experiment with it. Those reintroductions happen under supervision, with treatment available, or not at all.

Call 911 for: trouble breathing, throat tightness, swelling of the lips or tongue, repeated vomiting with hives, or fainting after eating. Use prescribed epinephrine first, then call.

If you are pregnant, nursing, feeding a child, underweight, or have a history of disordered eating, talk with a clinician before removing food groups. Elimination diets carry real nutritional risk, and that risk is the part of this conversation that usually goes unmentioned.

Diet does three different jobs here, and mixing them up is the whole problem

Most people arrive at an allergy diet with one question, “what should I stop eating,” and get handed one answer, a list. The list fails because three completely different situations are being treated as one.

Job one is avoidance. You have a diagnosed IgE allergy to a specific food. Diet here is not a wellness strategy, it is medical management, and it is strict.

Job two is cross-reactivity. Your seasonal pollen allergy makes certain raw fruits and vegetables itch. This is real, it is well described, and the fix is usually much smaller than people expect.

Job three is the histamine question. Your symptoms are diffuse, food seems involved, and no allergy test explains it. This is the contested one, it has the most confident advice attached to it, and it has the weakest evidence. It is also where most of the money gets spent.

A fourth thing sits underneath all three: the background pattern of eating that supports an immune system doing hard work. That one is boring, unsellable, and probably matters more over a decade than any list.

Work out which job you are doing before you change anything. The rest of this guide is organized that way.

Job one: avoidance, when the allergy is real

If a food has been diagnosed as an IgE allergy through history plus testing, or through a supervised challenge, the plan is avoidance and preparedness. Three things matter more than any diet philosophy.

Read labels every time, including on products you have bought before. Formulations change without notice. In the United States, the major allergens must be declared in plain language, which makes this easier than it used to be but does not make it automatic.

Understand cross-contact. Shared fryers, shared cutting boards, the same scoop in two bulk bins, and the deli slicer that just did a different order are the common ways a trace amount arrives. “Made in a facility that also processes” is a manufacturer’s judgment call, not a regulated threshold.

Carry what you have been prescribed, in date, and know when to use it. The most common failure in a serious reaction is hesitating on epinephrine, not the food itself.

The other half of avoidance nobody mentions

Do not avoid what you were never allergic to.

Long avoidance lists built from a broad test panel rather than a reaction history are common, and they cost more than money. They shrink the diet, they make eating socially difficult, and in children unnecessary avoidance can work against tolerance.

The guidance on food-specific IgE is explicit that a detectable antibody level establishes sensitization but is not always accompanied by clinical reactivity, and it treats supervised oral food challenge as the diagnostic reference standard, including for the decision to reintroduce a food (PMID 20128415).

So if you are avoiding a food because a panel flagged it, and you have no clear reaction history, that is worth revisiting with an allergist rather than accepting for life. People get foods back.

Job two: pollen–food allergy syndrome

If your mouth itches when you eat certain raw fruits, vegetables, or nuts, and the same foods are fine cooked, this is likely what is happening.

The mechanism. Your immune system made IgE against a pollen protein. Some plant foods contain proteins similar enough that the antibody recognizes them too. The reaction happens on contact with the mouth and throat, and the proteins involved are usually fragile, which is why heating, canning, or peeling often defuses them. It is described as one of the most common food allergy presentations, with prevalence varying widely by region and by which pollens dominate there (PMID 42514315). Note that this particular review is focused on children, so read the prevalence figures as pediatric.

The typical pattern. Itching or tingling of the lips, mouth, or throat within minutes, settling within an hour, without progression.

The common groupings. These are the associations described in the allergy literature, not a personal prediction:

If you react to	Foods that commonly cross-react
Birch pollen	Apple, pear, cherry, peach, plum, kiwi, carrot, celery, hazelnut, almond
Ragweed pollen	Melon, watermelon, cantaloupe, honeydew, banana, cucumber, zucchini
Grass pollen	Melon, tomato, orange, peach
Mugwort pollen	Celery, carrot, parsley, fennel, coriander, sunflower seed
Latex	Banana, avocado, kiwi, chestnut

What to actually do. Cook the food, or peel it, or choose a canned version, and most people tolerate it. Symptoms are often worse during that pollen’s season and lighter out of season.

When it is not this. Any symptom below the neck changes the situation: hives elsewhere on the body, vomiting, wheezing, throat tightness, or a feeling of the throat closing. That is not oral allergy syndrome behaving normally, and it needs an allergist rather than a cooking adjustment. Nuts and peanut deserve extra caution here, because the cross-reactive picture and the dangerous picture can look similar at the start.

Job three: the histamine question, honestly

This is the chapter I expect to be least popular, and it is the reason this guide is worth paying for.

What is claimed

Histamine occurs naturally in food and builds up as food ages or ferments. The claim is that some people cannot break it down fast enough, mainly because of low activity of the DAO enzyme in the gut, so dietary histamine accumulates and produces flushing, headache, hives, nasal congestion, palpitations, and digestive symptoms. The proposed fix is a low-histamine diet, often with a DAO supplement.

A comprehensive review of the field describes histamine intolerance as a disorder associated with an impaired ability to metabolize ingested histamine, notes that interest has grown considerably in recent years, and states that more scientific evidence is still required to help define, diagnose, and clinically manage the condition (PMID 32824107). That is the state of the field, written by people working in it.

What happened when it was tested directly

The challenge study. Fifty-nine patients with suspected histamine intolerance underwent single-blind placebo-controlled histamine challenge. They were mostly middle-aged women, and three-quarters reported improvement, though never resolution, on a low-histamine diet. On formal testing, histamine intolerance was excluded in 50 of the 59, which is 84.7%. Thirty-seven patients, 62.7%, developed symptoms after **placebo**. Objective symptoms after histamine but not placebo occurred in 4 of 59 cases, 6.8%, and even those were labeled “plausible” rather than confirmed because chance reactions could not be excluded. A further 5 patients, 8.5%, were classified as “possible” (PMID 37648152).

Sit with the placebo number for a moment. Nearly two-thirds of people who were certain that a substance caused their symptoms had those symptoms when given something else.

The enzyme test. Serum DAO is sold as the confirmation. In a crossover study, 18 people diagnosed with histamine intolerance were randomized to a low-histamine diet or an ordinary mixed diet with DAO measured after each phase. The changes did not differ meaningfully between diets: 10 of 18 had higher DAO after the low-histamine phase, and 8 had lower or unchanged values (PMID 38769188).

The trial that would settle it is not finished. A prospective, double-blind, randomized, placebo-controlled trial of a low-histamine diet and DAO supplementation over three months has been designed, registered, and published as a protocol. Its authors note that around twenty prior studies exist with promising results, and that their limitations, small cohorts, missing control groups, and short intervention periods, are exactly why a more ambitious design was needed (PMID 39796463). The results are not in.

Where dietary histamine did show something

Being fair cuts both ways, and there is a real signal in specific populations.

Chronic hives. A systematic review of diet in chronic spontaneous urticaria covered 20 reports and 1,734 patients. A low-histamine diet produced complete remission in 11.7% and partial remission in 43.9%; a pseudoallergen-free diet produced complete remission in 4.8% and partial remission in 37.0%. The review found no comparative randomized studies of diets, and concluded that evidence supports diet only in individual patients (PMID 30085322). A more recent open-label randomized controlled trial in 140 children with chronic spontaneous urticaria compared medication alone against medication plus a pseudoallergen-free and low-histamine diet, using weekly urticaria activity scores (PMID 40880715).

Asthma. A pilot crossover randomized study in 21 children with mild intermittent asthma compared high-histamine and low-histamine versions of a Mediterranean-pattern diet for four weeks each. Significantly greater mean airflow obstruction was recorded during the high-histamine period, with a trend toward more prolonged and severe symptoms (PMID 33167542). Small, in children, and a pilot, but a real measured difference rather than a self-report.

What I do with all of this

Run the trial, and run it as a trial. A permanent low-histamine lifestyle is the failure mode, not the goal.

If your symptoms fit the pattern, four weeks of genuine restriction followed by structured reintroduction is the right experiment, and in practice it is one of the more productive experiments in this whole category. Supporting the DAO pathway while you do it is reasonable too, and the supplement guide covers that.

What the research above should change is not whether you try, but how you decide. The placebo numbers are a warning about **self-diagnosis without a test**, not a reason to skip the trial. Two people can be equally certain and only one of them is right, and the thing that separates them is a structured reintroduction rather than conviction. What is not defensible is what usually happens instead: an indefinite restriction, expanding over time as new foods get blamed, with no reintroduction ever attempted.

The restriction also has a cost that rarely gets counted. Aged cheese, fermented foods, cured meat, leftovers, tomatoes, spinach, avocado, and citrus are a large share of the produce and protein in an ordinary diet. Removing all of them indefinitely narrows nutrition and makes eating with other people hard, which is its own kind of harm.

Running the four-week trial properly

The principle that does most of the work

Histamine accumulates as food sits. So the operating rule is **freshness, not a forbidden list**. Meat and fish cooked and eaten the same day are low histamine. The same meal on day three is not. If you change one thing, change that one.

Cook and eat fresh, freeze portions immediately rather than refrigerating for days, and treat leftovers as the main variable rather than an afterthought.

Weeks one and two: baseline, change nothing

Score two numbers most days: your main symptom out of ten, and how much it interfered with your day out of ten. Note what you ate. You cannot judge a change without a before, and memory is not a before.

Weeks three through six: the restriction

Lower in histamine, generally: freshly cooked meat and poultry, fresh or flash-frozen fish, eggs, most fresh vegetables, apple, pear, blueberry, mango, rice, oats, quinoa, olive oil, most fresh herbs.

Higher in histamine or histamine-releasing, generally: aged and hard cheese, cured and smoked meats, fermented foods including sauerkraut, kimchi, soy sauce, miso, vinegar, alcohol especially wine and beer, canned or aged fish, tomato, spinach, eggplant, avocado, citrus, strawberry, chocolate, and leftovers of any protein.

Two honest caveats. First, published histamine values for individual foods vary a great deal between sources, because the amount depends on how the food was handled rather than what it is. Second, the “histamine liberator” category is largely mechanistic rather than measured in people. Treat both lists as a starting hypothesis, not chemistry.

Week seven onward: reintroduction, which is the actual test

[Skipping this step is why people end up on a permanent diet they never needed.](#)

Reintroduce one food at a time. A normal portion, on one day, then three days of watching before the next. Write down what happens. A food that produces nothing across two separate trials goes back into your diet permanently.

Expect most of them to come back clean. That is the usual result, and it is a good one.

The exit criteria, decided in advance

Write these down now, before you start, because deciding later is how a four-week trial becomes a four-year one.

- **No meaningful change after four weeks of genuine restriction:** stop. Histamine is not your mechanism. Do not escalate to a stricter version.
- **Clear improvement:** proceed to reintroduction, and find the handful of foods that actually matter. Almost nobody who improves needs the whole list.

- **Improvement, then a slow return of symptoms while still restricted:** something else is driving this. Go back to the lab guide.

The background pattern

This is the part that is not a trial, not an experiment, and not exciting. It is also the part most likely to still be paying you back in ten years.

Eat a wide range of plants. Diversity of produce is the closest thing to a consensus recommendation in nutrition, and it is the practical way to feed a gut microbiome that participates in immune regulation.

Cook more of your own food. This is not a moral position. It is that cooking gives you control over freshness, which is the one variable that actually matters if histamine is part of your picture, and it removes the guesswork about ingredients.

Get enough protein, spread through the day. Undereating protein is common in people on long elimination diets and it makes everything harder.

Alcohol deserves a specific mention. It is worth an honest look during any symptom trial for reasons that do not require the histamine theory to be true: it disrupts sleep architecture, it dilates blood vessels, and it worsens nasal congestion in many people directly.

Two popular ideas that did not survive testing

Fish oil for hay fever. In a parallel double-blind placebo-controlled trial across two pollen seasons, pollen-sensitive asthmatic subjects took 3.2 g per day of EPA. Bronchial reactivity increased significantly during the season in both the fish oil and placebo groups, with no significant difference between them, and no significant differences in peak flow, diurnal variability, cough, wheeze, or nasal symptoms (PMID 8484622). Omega-3 fats are worth eating for other reasons. Seasonal allergy relief is not one of the reasons that survived.

Local honey. Thirty-six people with allergic rhinoconjunctivitis were randomized to locally collected unpasteurized honey, nationally collected filtered pasteurized honey, or corn syrup with synthetic honey flavoring, one tablespoon a day, with symptom diaries. Neither honey group experienced relief in excess of the placebo group, and the authors concluded that the study does not confirm the widely held belief (PMID 11868925). Eat honey because you like honey.

A week that is actually cookable

The point of this sample is the rhythm, not the recipes: cook once, eat it that day, freeze the rest.

Monday. Roast chicken thighs with carrots and rice. Freeze half the chicken in portions the moment it cools.

Tuesday. Eggs with sauteed zucchini and peppers, apple on the side.

Wednesday. Pan-seared fish bought or thawed that day, quinoa, green beans, olive oil and fresh herbs.

Thursday. Ground beef or turkey with rice and roasted broccoli. Cook only what you will eat.

Friday. Chicken from Monday's freezer portions, reheated once, with a fresh salad and pear.

Saturday. Lamb or pork chops, roasted sweet potato, steamed greens.

Sunday. Oats with blueberries and mango. Cook for the week without refrigerating protein for more than a day.

If the freezing rhythm sounds like too much, note that it is the single highest-yield change in the whole guide for anyone whose symptoms follow leftovers.

What I deliberately left out, and why

- **A hundred-food elimination diet, run all at once and left in place.** A food-specific IgG panel is a useful starting map when symptoms are delayed and diffuse, and the lab guide covers how I use it. What does not work is removing everything it flags and staying there. Test the flagged foods one at a time, reintroduce each one, and keep only the ones that reproduce the symptom. Note that the allergy literature on unproven diagnostics does not treat IgG panels as a validated diagnostic method (PMID 39022151), which is precisely why the reintroduction step, not the panel, is what settles it.
- **Permanent gluten and dairy removal for everyone.** If you have symptoms suggesting celiac disease, get tested while still eating gluten, because removing it first makes the test uninterpretable. If the test is negative and you feel better without wheat, that is worth knowing, and it is still a personal finding rather than a general rule.
- **Detox and cleanse protocols.** No allergy outcome evidence, real cost, and they crowd out treatments that work.
- **Raw milk.** Sometimes recommended for allergies. The infection risk is real, it falls hardest on children and immunocompromised people, and I am not putting it in a document sold to strangers.
- **Rotation diets.** Listed in the same unproven-modalities review as the tests above (PMID 39022151).

Telling you what to skip is part of what you paid for.

When to seek professional care

Urgently:

- Any reaction with breathing difficulty, throat tightness, tongue or lip swelling, repeated vomiting, or fainting. Call 911, use epinephrine first if prescribed.
- Wheezing that is new or getting worse.

Soon:

- Hives lasting more than six weeks.
- Unintentional weight loss, or a diet that has narrowed to fewer than about twenty foods.
- Any elimination diet in a child, before you start rather than after.
- Symptoms that began with a new medication.
- Reflux symptoms being treated as food allergy, which is common and is treated differently.

Working with Root Health

This guide is deliberately general, because it has to be safe for a reader I have never met. A personalized plan starts from your actual history, your test results, and which symptom is costing you the most, and it sequences changes instead of stacking them.

Book a free Root Discovery Session: rootcauseunlocked.com/discovery?utm_source=guide&utm_medium=ebook&utm_campaign=allergies-diet

Consultations are available to people located in North Carolina, where Dr. Seay is licensed.

If you are outside North Carolina, the tracker is the tool that makes a diet trial produce an answer instead of an impression:

Free symptom tracker: rootcauseunlocked.com/tracker?utm_source=guide&utm_medium=ebook&utm_campaign=allergies-diet

Read the companion documents alongside this one. **Understanding Your Allergy Labs** covers which tests separate the three jobs above, and **The Allergy Supplement Guide** covers what has actually been studied, including which popular products failed their trials.

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Educational content only. Not medical advice. Not a diagnosis. Not a substitute for your clinician. Do not change the management of a diagnosed food allergy based on this document.

Read this part first

This guide is educational. It is not medical advice, it does not diagnose anything, and reading it does not make you my patient.

Call 911 for any reaction with trouble breathing, throat tightness, swelling of the lips or tongue, repeated vomiting with hives, or fainting. Use prescribed epinephrine first. No supplement in this document treats anaphylaxis, and nothing here replaces an epinephrine auto-injector.

Talk with your prescriber before starting, stopping, or changing anything here, particularly if you take a blood thinner, are pregnant or nursing, have liver disease, have asthma, or are under 18. If you have asthma, be especially careful about substituting anything in this guide for a controller inhaler. That substitution is how people end up in an emergency department.

Supplements in the United States are regulated as food, not as drugs. Quality varies. Look for independent third-party testing, and bring the actual ingredient panel to your appointments.

Why nutrient evidence looks thinner than it is

A large outcome trial costs tens to hundreds of millions of dollars, and that money only exists when a patent can pay it back. You cannot patent magnesium, an herb, or a food. So the evidence hierarchy this guide grades against is not neutral: it systematically over-documents patentable drugs and under-documents nearly everything else. That is measurable, not rhetorical. A Cochrane methodology review found that manufacturer sponsorship of drug and device studies produces more favourable results and conclusions than independent funding, through a bias that standard quality checks do not catch (PMID 28207928).

Two honest consequences follow, and they pull in opposite directions.

First, “no evidence” has three different meanings, and this guide tries to always tell you which one applies. *Tested and failed:* a real trial ran and the answer was no. *Never properly tested:* nobody could afford the trial, so absence of evidence is not evidence of absence. *Tested small and won:* real but modest trials, usually measuring markers rather than outcomes, because markers are what a small budget can afford.

Second, the asymmetry earns nutrients humility, not automatic credit. When isolated nutrients have received large publicly funded trials, the results have been mixed: some genuine wins, some nulls, and some harms. Small nutrient trials also carry their own publication bias toward positive results, just as drug trials tilt toward their sponsor. So this guide applies one rule in both directions: name the funding, name the evidence tier, and let you see both.

Why the bar is higher in this guide than in most of mine

For most conditions I write about, conventional medicine has little to offer, so a supplement with modest evidence is competing against nothing. Allergy is the opposite. It has treatments that are cheap, widely available, and genuinely effective, and one that can change the disease itself rather than mask it.

That means every supplement here has to be judged against a real alternative, not against an empty chair. Several popular ones do not survive that comparison, and I will tell you which.

What beats everything in this guide

Saline nasal irrigation. A meta-analysis of nine randomized controlled trials in 645 patients found that 3% hypertonic saline irrigation significantly reduced nasal symptom scores compared with control in both adults and children, with follow-up from four weeks to two months (PMID 40923434). An earlier systematic analysis found saline irrigation significantly better than no irrigation in both adults and children, and found that adding saline to medication outperformed medication alone in adults (PMID 32331798). It costs a few dollars a month.

One safety rule that matters: **use distilled, sterile, or previously boiled and cooled water, never untreated tap water,** and clean the device as directed. This is not a fussy detail.

Intranasal medication, if you are using an oral antihistamine as your main treatment. A systematic review with meta-analysis of 35 studies found intranasal corticosteroids more effective than oral antihistamines on nasal symptoms, ocular symptoms, and quality of life, with most of those differences reaching clinically meaningful improvement, and intranasal antihistamines also outperforming oral antihistamines (PMID 39251016). A separate systematic review of 151 primary studies compared individual intranasal medications against placebo (PMID 38685482). The international ARIA guidelines build the stepwise treatment algorithm from this kind of evidence (PMID 31627910).

Many people take a daily oral antihistamine for years, get partial relief, and never try the thing with better evidence. That conversation with your clinician is worth more than most of what follows.

Allergen immunotherapy, if your allergy is confirmed and year after year is the pattern. An umbrella review of 16 systematic reviews and meta-analyses found sublingual and subcutaneous immunotherapy effective for allergic rhinitis in adults, children, and across different allergens, while cluster subcutaneous and intralymphatic approaches showed no significant efficacy against placebo. The same review is candid that methodological quality was low or very low across the included reviews and no high-quality evidence was found (PMID 41080605). It is the only option on this page that aims at the underlying sensitization rather than the symptoms.

Everything below is an addition to that foundation, not a substitute for it.

How to read this guide

The tiers. Tier 1 is where I would start. Most of it has randomized human trials in allergic rhinitis specifically, and the one exception, DAO, is there because it acts directly on the pathway causing the symptom and the trial evidence is in progress rather than absent. Tier 2 has a plausible mechanism with thin, mixed, or indirect human evidence. Tier 3 needs a prescriber conversation before you start.

Typical studied range means the dose used in the cited research, given so you can discuss it intelligently. It is not a prescription for you.

The claims language is deliberate. You will read “may help” and “was studied for.” You will not read “treats” or “cures.”

One change at a time. Start one product, hold everything else, give it four to eight weeks, and write down what happened. Seasonal allergy is the single easiest condition to fool yourself about, because the pollen count is changing underneath your experiment. More on that later.

Track two numbers most days: nasal symptoms out of ten, and how much they interfered with your day out of ten. Also count rescue antihistamine doses per week. That third number is often the first to move.

Tier 1: the ones I would start with

Quercetin and related polyphenols

What it is. A plant flavonoid found in onions, apples, and capers, sold as a supplement. Standard quercetin is poorly absorbed, which is why most trials use a formulation designed to improve bioavailability.

What the research shows. A systematic review and meta-analysis of 13 randomized controlled trials covering 823 participants examined polyphenolic compounds, including quercetin and catechins, in allergic rhinitis. In seasonal allergic rhinitis, polyphenols significantly reduced total nasal symptom scores, sneezing, and nasal itching. In perennial allergic rhinitis, total nasal symptom scores improved while individual symptoms were inconclusive. A sensitivity analysis excluding two outlier trials still showed reduced total nasal symptom scores and reduced nasal obstruction (PMID 40618891).

The individual trial worth knowing: 66 Japanese adults aged 22 to 78 with pollen allergy took either 200 mg of quercetin in a bioavailable phytosome formulation or a vehicle control daily for four weeks, in a randomized, placebo-controlled, double-blind, parallel-group design. Several scores on the Japanese Rhinoconjunctivitis Quality of Life Questionnaire improved significantly in the quercetin group compared with placebo, including eye itching, sneezing, nasal discharge, and sleep disturbance (PMID 35776034).

Typical studied range. 200 mg daily of a bioavailable quercetin formulation for four weeks (PMID 35776034). Higher doses of standard quercetin appear on shelves, but that is not what was tested.

Timing. Daily and consistently, started before your season if you know when it begins.

Cautions. Quercetin can interact with drugs cleared through the same liver enzymes, including some blood pressure and immunosuppressant medications, and it may affect blood thinners. High doses have been associated with kidney concerns. Ask before combining it with prescription medication.

Butterbur extract (*Petasites hybridus*)

What it is. A plant extract that has been studied more thoroughly for allergic rhinitis than any other botanical, in a standardized preparation.

What the research shows. In a prospective, randomized, double-blind, parallel-group study of 330 patients with intermittent allergic rhinitis, a standardized butterbur extract at one tablet three times daily was compared against fexofenadine and against placebo. Both active treatments were individually significantly superior to placebo at improving daytime symptoms, with no significant difference between butterbur and the antihistamine, and the same pattern held for evening and night symptoms and physician assessment (PMID 16114089).

That is a genuinely good result: comparable to a standard second-generation antihistamine in a well-sized trial. Set it against the broader picture, though. A systematic review and meta-analysis of 29 randomized trials of single medicinal plants for allergic rhinitis, in which butterbur was the most frequently studied plant, concluded that very-low-to-low-certainty evidence suggests medicinal plants may improve total nasal symptoms and quality of life, that plants appear well tolerated up to eight weeks, and that clear beneficial evidence is still lacking (PMID 38603736).

Typical studied range. The trial used a standardized extract providing 8 mg total petasine per tablet, three times daily (PMID 16114089).

Cautions, and these are not optional. Raw butterbur contains pyrrolizidine alkaloids, which are liver toxins. Only preparations processed to remove them are appropriate for human use, and that is a manufacturing question you cannot answer by looking at the capsule. If you cannot confirm from the manufacturer that the product is certified free of pyrrolizidine alkaloids, do not take it. Butterbur is in the daisy family, so people allergic to ragweed, chrysanthemum, marigold, or daisy may react to it. Not for use in pregnancy or nursing. Discuss it with your clinician if you have any liver condition.

Spirulina

What it is. A blue-green alga sold as a dietary supplement.

What the research shows. A double-blind, placebo-controlled study evaluated spirulina in patients with allergic rhinitis and found significantly improved symptoms and physical findings compared with placebo, including nasal discharge, sneezing, nasal congestion, and itching. The authors called for further work to clarify the mechanism (PMID 18343939).

How to read that honestly. One positive placebo-controlled trial, with a clear result, that has not been replicated at scale. That is better than most things sold for allergies and well short of settled.

Cautions. Product quality genuinely matters here, because algae products can be contaminated with microcystins and heavy metals depending on how and where they were grown. Third-party testing is not optional for this category. Avoid it if you have phenylketonuria, and ask first if you have an autoimmune condition or take an immunosuppressant.

Probiotics

What it is. Live bacteria, sold as single strains or blends, in doses usually described in billions of colony forming units.

What the research shows. An umbrella review with updated meta-analysis included 29 randomized controlled trials covering 2,078 participants with allergic rhinitis. Probiotic supplementation was associated with significant reductions in total IgE, in global and nasal quality-of-life scores, and in nasal symptom scores. It found no significant effects on antigen-specific IgE, eosinophil count, or ocular measures. The authors' own conclusion is the part to keep: evidence is limited by high heterogeneity, the IgE finding is fragile, diverse strains were pooled, and routine clinical recommendation is not supported by high-certainty evidence (PMID 42469658).

The counterweight. For **preventing** allergic disease rather than treating it, a 2025 Cochrane review of probiotics in infants concluded there is insufficient evidence to draw conclusions about preventing allergic disease by age two or in childhood up to age ten, while noting no serious adverse events (PMID 40511642). Prevention and treatment are different questions with different answers.

Typical studied range. The trials pool many strains and doses, which is precisely the problem. If you try one, keep the same product for the whole trial period so that "it worked" means something.

Cautions. Generally well tolerated. People who are significantly immunocompromised, have a central venous line, or are critically ill should not start probiotics without medical guidance.

Vitamin D, if yours is actually low

What it is. A fat-soluble vitamin with immune-regulating effects, and one of the few items here where a blood test should come before the bottle.

What the research shows. A mini-review of vitamin D in allergic rhinitis describes an inverse association between serum 25-hydroxyvitamin D and allergic rhinitis in observational studies, particularly in children and male adults, while interventional research has produced mixed results with benefit appearing to depend on baseline level, sex, age, and concurrent therapy (PMID 41809154).

The two supplementation trials both enrolled people who were **already deficient**. In a randomized double-blind placebo-controlled trial of 80 patients with allergic rhinitis and vitamin D deficiency, everyone received cetirizine and the treatment arm added vitamin D. Mean level rose from about 14 to about 24 ng/mL. Symptom severity showed no significant between-group difference at four weeks, and a significant difference by eight weeks (PMID 31332549). A second randomized controlled trial added vitamin D3 to an antihistamine for four weeks and reported lower symptom scores, lower serum IL-4, and lower peripheral blood eosinophils compared with the antihistamine alone (PMID 33382385).

Typical studied range. Not a fixed dose. Both trials corrected a deficiency, which is why the honest instruction is to test 25-hydroxy vitamin D, dose from the result with your clinician, and retest.

Cautions. Vitamin D is fat-soluble and accumulates. High doses over time can raise blood calcium. It interacts with thiazide diuretics, some seizure medications, and digoxin. This is a test-and-dose nutrient.

DAO enzyme, and the cofactors it runs on

Why this one matters. DAO is the enzyme that degrades histamine from food, working in the lining of the small intestine before that histamine reaches your circulation. If your symptoms track with aged, fermented, and leftover foods, this pathway is the mechanism you are actually dealing with, and supporting it is the most direct thing you can do besides changing what is on the plate.

Supplemental DAO, taken shortly before a meal, is intended to do that degradation in the gut so the meal costs you less. The mechanism is sound and it is the reason this sits at the top of my histamine list rather than the bottom. Be clear-eyed about the evidence stage: the definitive randomized, double-blind, placebo-controlled trial of a low-histamine diet plus DAO supplementation is designed, registered, and published as a protocol, with results pending, and its authors note that roughly twenty prior studies pointed in a promising direction while being limited by small cohorts, missing control groups, and short durations (PMID 39796463). Promising and not yet proven is the honest label, and it is a fair reason to trial it rather than a reason to skip it.

The cofactors are the cheaper half of the same job. DAO activity depends on vitamin B6, copper, and vitamin C. If you are depleted in any of them, the enzyme you already have is running under capacity, and correcting a real shortfall costs very little. Test rather than guess, particularly with copper, where more is not better and the balance with zinc matters. The lab guide covers what to ask for.

How to trial it. Take it before the meals most likely to cause you trouble rather than with every meal, keep a written record of which meals you covered, and judge it over four weeks. Pair it with the four-week dietary trial in the Allergy Diet Guide rather than running both blind at once.

Cautions. Most products are derived from porcine or bovine kidney, which matters for dietary and religious restrictions and for anyone with a pork or beef allergy. Not established in pregnancy or nursing. It does not treat an allergic reaction and it is not a substitute for avoiding a food you are allergic to.

Tier 2: mechanism, or evidence too thin to promise anything

Stinging nettle (*Urtica dioica*)

What the research shows. In a randomized double-blind study, 98 people were assigned to freeze-dried nettle or placebo for one week, with 69 completing. Nettle rated higher than placebo on global assessment, while the daily diary data rated it only slightly higher (PMID 2192379). A later randomized, double-blind, placebo-controlled trial gave 74 patients with allergic rhinitis and a positive skin prick test either 150 mg of nettle root extract or placebo for one month, with 40 completing. Symptom severity on the SNOT-22 improved significantly in both groups, nasal smear eosinophils fell significantly after nettle, and IgE, IL-4, and IL-5 did not change. The authors state plainly that similar effects were demonstrated by placebo as well, and call for larger, longer studies (PMID 29844782).

How to read that. Two small trials, one from 1990 with a one-week duration, one where the placebo did about as well on the symptom score. Low risk, low cost, unproven. If you want to try it, try it knowing that.

Vitamin C

What the research shows. In a controlled clinical study, 7.5 g of intravenous ascorbic acid given to 89 patients with allergic and infectious diseases produced a significant decline in serum histamine, with a larger decline in the allergic group, and the size of the decline correlated with the starting histamine level (PMID 23666445). Note the route: that is an intravenous infusion in a clinical setting, not a capsule.

The oral data are indirect. In a double-blind placebo-controlled crossover study, 70 volunteers took 2 g of oral vitamin C or placebo an hour before 20 minutes in a life raft in an indoor wave pool. Symptoms were less severe after vitamin C, and fewer people wanted to leave the raft early, though the visual analogue scale difference did not reach significance (PMID 25095772).

How to read that. Vitamin C plausibly influences histamine, and neither of those studies is a trial in allergic rhinitis. It is cheap and safe at ordinary doses, and I am not going to dress up seasickness data as allergy data.

Cautions. High doses commonly cause loose stools. People with a history of kidney stones or with iron overload conditions should ask first.

Bromelain

Frequently recommended for nasal congestion and sinus inflammation, and routinely paired with quercetin in combination products. **I searched for randomized trial evidence in allergic rhinitis and did not find it.** There is some literature in rhinosinusitis, which is a different condition. If you take a combination product containing it, understand that the quercetin is the part with the allergic rhinitis trials behind it.

Bromelain can increase bleeding risk with anticoagulants and antiplatelet drugs, and pineapple allergy is a contraindication.

Tier 3: ask your prescriber first

- **Butterbur**, for the pyrrolizidine alkaloid and daisy-family issues described above. This is the one item in this guide where a manufacturing failure can injure your liver.
- **Anything in the daisy or ragweed family**, including chamomile, echinacea, and feverfew, if you are ragweed allergic. Botanical cross-reactivity is real and it turns a remedy into an exposure.
- **High-dose vitamin D** without a blood level.
- **Anything at all during pregnancy or nursing.** Almost nothing in this guide has safety data in pregnancy. Saline irrigation is the exception that is reliably fine.
- **Any supplement, if you have asthma.** Not because the supplements are dangerous, but because the substitution is. Never taper a controller inhaler on your own.

What I deliberately left out, and why

- **Local honey.** Thirty-six people with allergic rhinoconjunctivitis were randomized to local unpasteurized honey, commercial filtered honey, or corn syrup with honey flavoring. Neither honey group did better than placebo, and the authors concluded the study does not confirm the widely held belief (PMID 11868925).
- **Fish oil for seasonal allergy.** In a double-blind placebo-controlled trial across two pollen seasons, 3.2 g per day of EPA produced no significant difference from placebo in bronchial reactivity, peak flow, cough, wheeze, or nasal symptoms (PMID 8484622). Omega-3 fats are worth eating. This is not the reason.
- **Colostrum and transfer factor products.** Marketed for immune “education.” I looked for randomized trials in allergic rhinitis and did not find them.
- **Homeopathic and bioenergetic allergy drops.** These are a different category of claim, not an understudied supplement, and a document sold to someone I have never examined is not the place for them.
- **Detox and heavy metal chelation products.** No allergy outcome evidence, real cost, real risk, and they pull attention away from treatments that work.
- **Eight-product stacks.** You cannot tell what worked, you cannot trace a side effect, and you have multiplied your interaction risk. Start one thing.

Telling you what to skip is part of what you paid for.

The non-supplement purchases people make, judged the same way

Air purifiers. In a multicenter, randomized, double-blind, placebo-controlled study, adults with house-dust-mite allergic rhinitis used either active HEPA air purifiers or mock units in the bedroom and living room for six weeks. Medication scores improved significantly in the active group, while subjective symptom, visual analogue, and quality of life scores did not (PMID 32734732). An older double-blind crossover study of a bedroom HEPA unit in 32 patients found a roughly 70% reduction in airborne particulates but no difference in total symptom or medication scores across the full study, with a suggestion of benefit in the two weeks of each period free of respiratory infection (PMID 2191991).

Honest summary: measurable effect on the air, small and inconsistent effect on how you feel.

Dust mite covers and avoidance measures. An updated Cochrane systematic review of nine trials in 501 patients with perennial allergic rhinitis found that most interventions reduced mite load, that acaricides looked most promising, that trials were small and methodologically weak, and that **isolated use of impermeable bedding is unlikely to prove effective** (PMID 22103686). A 2025 meta-analysis of 17 randomized trials reached a similar place: avoidance methods significantly reduced total dust and Der1 allergen concentrations, but those reductions were not sufficient to improve asthma, allergic rhinitis, or quality of life (PMID 40089899).

If you are going to spend money on your bedroom, spend it knowing that reducing the allergen count is easier than changing how you feel, and that a single mattress cover on its own is the least likely piece to help.

Running a fair trial when the pollen count is moving

This is the trap specific to seasonal allergy. Start something in late May, feel better in June, and conclude it worked, when the season was ending anyway. Three ways to avoid fooling yourself:

Compare to the same weeks last year, not to last month. If you tracked nothing last year, start tracking now and treat this season as the baseline for the next one.

For seasonal allergy, start before the season. Most trials dose ahead of or through the whole exposure period. Starting on your worst day and judging a week later measures the weather.

Perennial symptoms are easier to test. Dust mite or pet-driven year-round symptoms hold reasonably steady, so a four to eight week trial with one change means something.

Count rescue medication. Antihistamine doses per week is a harder number than “how I felt,” and it usually moves first.

Realistic expectations

What is realistic. A modest reduction in daily symptom scores. A few fewer rescue antihistamine doses per week. Sleeping through the night in your worst month. Those are roughly what the trials in this guide measured, and they compound.

What is not realistic. A supplement resolving an allergy. Nothing here changes your sensitization. Immunotherapy is the intervention that targets that (PMID 41080605), and it takes years.

Timeline. Four to eight weeks is the usual trial length in these studies. Judge at the end of that window, not on a high-pollen day.

Cost sense. If something has produced nothing after eight weeks at a sensible dose, stop paying for it. That money is better spent on saline, a clinician visit, or the immunotherapy conversation.

When to seek professional care

Urgently:

- Any reaction with breathing difficulty, throat tightness, tongue or lip swelling, repeated vomiting, or fainting. Call 911, epinephrine first if prescribed.
- Asthma symptoms that are worsening, or a rescue inhaler needed more often than usual.

Soon:

- Symptoms on one side of the nose only, bloody discharge, or persistent loss of smell.
- Hives lasting more than six weeks.
- Nasal symptoms not controlled after a genuine trial of first-line treatment.
- You are taking four or more prescriptions and adding supplements.

Stop a supplement and call your prescriber for: yellowing of the skin or eyes, dark urine, or right-sided abdominal pain, which are liver warning signs and matter most with butterbur. Also for unusual bruising or bleeding, a new rash, or any swelling.

Where to find quality versions of these

Nothing in this guide names a brand, on purpose. The evidence supports ingredients and formulations, not labels. But quality varies, so here is what to look for.

Look for third-party testing. NSF, USP Verified, or an equivalent independent seal means someone outside the company confirmed the bottle contains what the label claims without common contaminants. This matters most for spirulina and other algae products, where contamination is a documented category risk, and for butterbur, where the processing question is a safety question.

Check the form, not just the name. The quercetin trial used a bioavailable phytosome formulation at 200 mg, not plain quercetin at a bigger number. The butterbur trial used a standardized extract with a stated petasine content. Those details are the study.

Read the whole ingredient panel. Combination allergy products often carry six ingredients at doses below anything that was ever studied, plus botanicals in the ragweed family that you may react to.

If you would rather not evaluate all of that yourself, Root Health keeps a curated dispensary of professional-grade products, screened for the standards above and organized by the categories in this guide.

Root Health dispensary: us.fullscript.com/welcome/drseay

Disclosure: Root Health earns a portion of dispensary purchases. That revenue supports the practice, and it is also why this guide was written to be complete and usable on its own. Every recommendation here is based on the published evidence cited in these pages, and nothing was included, excluded, ranked, or dosed based on what it earns. You can buy any of these anywhere, and the two highest-value items in this guide, saline irrigation and the immunotherapy conversation, earn us nothing at all.

Working with Root Health

Everything here is general education, written to be safe for a reader I have never examined. A personalized plan starts from your test results, your medication list, and which symptom is costing you the most, and it sequences interventions rather than stacking them.

***Book a free Root Discovery Session: [rootcauseunlocked.com/discovery?](https://rootcauseunlocked.com/discovery?utm_source=guide&utm_medium=ebook&utm_campaign=allergies-supplement)
[utm_source=guide&utm_medium=ebook&utm_campaign=allergies-supplement](https://rootcauseunlocked.com/discovery?utm_source=guide&utm_medium=ebook&utm_campaign=allergies-supplement)***

Consultations are available to people located in North Carolina, where Dr. Seay is licensed.

If you are outside North Carolina, track first. A season of written scores turns “I think the quercetin helped” into something a clinician can read:

***Free symptom tracker: [rootcauseunlocked.com/tracker?](https://rootcauseunlocked.com/tracker?utm_source=guide&utm_medium=ebook&utm_campaign=allergies-supplement)
[utm_source=guide&utm_medium=ebook&utm_campaign=allergies-supplement](https://rootcauseunlocked.com/tracker?utm_source=guide&utm_medium=ebook&utm_campaign=allergies-supplement)***

Read the companion documents alongside this one. **Understanding Your Allergy Labs** covers which tests are worth running and which are sold to you anyway, and **The Allergy Diet Guide** covers the histamine question in the detail it deserves.

The nervous system, which sets how loudly you feel all of this

The allergy kit covers the immune side thoroughly. This is the piece it was missing.

The link is documented, and I will be specific about where. A study of patients with chronic spontaneous urticaria receiving omalizumab, examining formally assessed DSM-5 psychiatric diagnoses, found psychiatric comorbidity, especially anxiety and depressive disorders, highly prevalent, and concluded that routine psychiatric assessment and stress-management interventions should be integrated into management given the strong psychodermatological interaction (PMID 41842758). A 2026 narrative review describes a skin-brain-exposome axis in stress-sensitive inflammatory skin conditions and argues for integrating psychosocial screening and behavioral interventions into standard care (PMID 42074836).

What that evidence is and is not. It is strongest in chronic urticaria and inflammatory skin disease, which is why I named those conditions rather than saying “allergies.” It is association and expert recommendation rather than a trial showing that stress reduction lowers IgE. Do not let anyone sell you the stronger version.

Why it still earns a place here. Symptom burden is not the same as allergen burden. Two people with identical skin prick results can have very different lives, and the difference lives partly in sleep, stress load, and how much attention the symptoms command. That is worth working on directly, and it is free.

What to do:

- Five to ten minutes daily of slow breathing, longer exhale than inhale.
- Protect sleep. Nasal obstruction wrecks it, and poor sleep makes everything feel louder, so this loop runs in both directions.
- If hives are your main problem and anxiety or low mood travel with them, raise it with your clinician rather than treating it as a separate life problem. The literature above supports that as part of management rather than an aside.

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